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Barriers and opportunities for involvement in health policy development of Somaliland nurses: Qualitative study

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ABSTRACT

Background: The nursing profession is the backbone of the healthcare system and the largest profession of healthcare workers globally. Nurses play a crucial role in providing healthcare services in the community. In recent years, nurses have improved their knowledge and skills but remained undervalued and unappreciated, with a poor public image.
Aim: This study was to investigate the degree of involvement of Somaliland nurses in health policy development.
Methods: A qualitative study design was adopted using in-depth interviews. A total of 13 interviews were conducted using open-ended questions. The data were analysed using an inductive reflective thematic analysis.
Results: One main theme and two sub-themes were identified. The sub-themes are (1) *navigating the barriers to nursing education and equity*, which emphasises the challenges in the proliferation of non-accredited private schools and policy engagement and the ambiguity and under-compensation in the nursing profession, and (2) *building a strong nursing ecosystem: education, leadership and advocacy*, which highlights the strengths of Somaliland nurses and the participants' recommendations.
Conclusion: The findings highlight the barriers to Somaliland nurses' involvement in health policy development, including unregulated nursing education, others filling the nursing role and low nursing salary, consistent with other studies. The study also reveals that the existence of academically qualified nurses, national nursing associations and public institutions led by nurses demonstrates competency and dedication to develop the nursing profession. Therefore, it is important to intentionally develop nursing leaders through mentorship and succession plans, which can help to remove current nursing challenges.

1. Introduction

Globally, it is estimated that 35 million healthcare workers are nurses, making nursing the largest profession in the world (World Health Organization, 2010). Since nurses primarily interact with patients and their families, they understand the patients' health needs, recognise how hospital environment affects the patients' condition, and consider the broader population context. A Delphi study conducted in East Africa identified the following factors that can enable nurse leaders to participate in health policy development: health policy knowledge, supporting role models and mentors, positive perception of their role as nurses, and structures and systems that facilitate their involvement (Shariff, 2014). Nurses are significant for health policy development due to several reasons. For instance, since they know their patients' health needs and preferences, they can effectively shape patient-centred policies. Moreover, as various health policies affect nursing practice, nurses can actively shape new policies (Smith et al., 2014).

The global Nursing Now campaign partnered with the International Council of Nurses (ICN) and the WHO to bolster and enhance strategic nursing leadership, as exemplified by the ICN's Global Nursing

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Leadership Institute. Several factors affect nurses' involvement in health policy, such as developing leadership skills, gaining knowledge in health systems and the health policy development process (Salvage et al., 2019).

Health policy concerns the decisions, plans and actions undertaken to achieve specific healthcare goals in a society, aiming to improve the quality of care that patients receive and enhance the patient safety (World Health Organization, 2019). Health policy has three phases: policy formulation, policy implementation, and policy reformation. However, even if nurses possess significant skills and knowledge and are obligated to implement health policy decisions, they are excluded from the policy decision-making process (Hajizadeh et al., 2021).

A study conducted in Iran found that nurses were moderately involved in health policy; they were, for example, responsible for crafting health policy reports on issues (Safari et al., 2020). Low-resource settings, such as Africa, have been associated with an imbalance of power between men and women, which is mainly related to the patriarchal system and is reflected in the healthcare system of such settings (Esterhuizen et al., 2013). A study conducted in Kenya found that the nursing profession is often perceived as a barrier rather than a key stakeholder because it is isolated from other medical fields and predominantly operates at the lower levels of the healthcare system, which in turn limits nurses' participation in Kenya's national policy-making (Juma et al., 2014). Nursing education also significantly impacts the roles nurses can play. Most East African nurses mainly have diplomas, and they focus more on patient care and clinical skills than on leadership, which limits their opportunities to hold leadership positions (Sharif, 2014).

Somaliland is a post-conflict, self-declared country that has undertaken significant efforts to rebuild its healthcare system after the civil war; however, it still struggles in providing effective, efficient and high-quality healthcare (Central Statistics Department (CSD) et al., 2020). It has also re-established nursing education, with the country now having 14 accredited institutes that train nurses in diploma, bachelor's degree programmes and some universities that offer postgraduate nurse education (National Health Professions Commission NHPC, 2024). According to Somaliland Nursing and Midwifery Association (SLMNA) unpublished data, in 2022 approximately 122 students graduated from all training institutions with a bachelor's degree in nursing in Somaliland of which work in private and public hospitals and in maternal and child health (SLMNA, 2022). With Somaliland rebuilding its healthcare system, its Ministry of Health Development (MoHD) is developing and implementing several health policies. Nurses are crucial for the successful implementation of these policies, and involving them in policy development supports accessible, available and affordable healthcare (Central Statistics Department (CSD) et al., 2020). Therefore, the primary objective of this study was to investigate the degree of involvement of Somaliland nurses in health policy development, and the secondary objectives were to explore which factors enable, which factors hindered their involvement in health policy development and to gather their suggestions on how to improve nurses' role in health policy involvement in Somaliland.

2. Methods

2.1. Design

This study used a qualitative design with an exploratory inductive approach, as it aimed to comprehensively understand the phenomenon under investigation (Creswell, 2022). The chosen approach is particularly appropriate, given the study's objective of outlining experiences and perspectives rather than testing pre-existing theories.

2.2. Setting

The study setting was Hargeisa, the capital city of Somaliland, a post-conflict country located in the Horn of Africa. The largest city in Somaliland, Hargeisa has a population of 1.2 million and is the focal point of all administrative activities (Kilcullen, 2019). After the civil war, health facilities were destroyed, trained nurses fled to refugee camps or overseas, and several nurses died, resulting in a shortage of nursing professionals. This study targeted public health facilities, local non-profit organisations, non-nursing decision makers of the Ministry of Health Development, the Somaliland Nursing and Midwifery Association, the National Health Professional Committee and three universities (the University of Hargeisa, Edna Adan University and Golis University).

2.3. Population and sampling

Participants were recruited from the target settings using purposive sampling and snowballing (Creswell, 2022). The inclusion criteria were registered nurses working in clinical, academic, and administrative roles with at least 3 years of professional experience, and other professionals, such as policymakers and those in leadership positions, were included. Nursing students, interns, and professionals who are not serving in nursing leadership roles were excluded from the data collection. We contacted the human resource department at the Ministry of Health and Development (MOHD), Universities in Hargeisa city, public health facilities, local non-profit organisations, the Somaliland Nursing and Midwifery Association, and the National Health Professional Committee, where we informed them about the study and asked if we could get in contact with nurses working at their departments. After interviewing one participant, we asked whether they could refer to someone they knew who could contribute to this study. In total, 14 potential participants were approached, 13 provided consent and were included, and 1 participant was unavailable. No new information emerged after ten interviews, and the research team determined that the depth and saturation of the collected data were sufficient to address the study objectives (Creswell, 2013).

2.4. Data collection

Thirteen open-ended interviews were conducted between February and March 2023, with support from a research team with extensive qualitative research experience. A semi-structured open-ended interview guide was developed and piloted by the first and third authors to ensure clarity (please see Table 2; sample of interview guide). The interview guide was developed by the research team, guided by the study's objectives and the topic's knowledge gap in the context. It included themes about (1) the role of Nurses in health policy development (2) the challenges nurses face in health policy involvement, (3) the factors that increase their involvement in health policy development, and (4) the suggestions on how to improve nurses' role in health policy involvement. Two pilot interviews were conducted with the participants and were transcribed and checked for completeness and depth of data relating to the study aim. Since no further changes were made to the interview guide, these two were included in the study. Before the interview, the participants were invited by the first author via telephone, and the study's aims and content were verbally explained. An appointment for an interview was agreed upon, and they were informed that they were free to withdraw from participation at any time without any consequences. The interviews were audio-recorded and lasted one hour and fifteen minutes; they were conducted in private spaces chosen by the participants in the Somali language. Twelve participants were interviewed face-to-face, while one was interviewed via telephone because of the long distance. The first and second authors translated and transcribed the interviews verbatim into English, and the third and last authors quality-checked that the translation was adequate. It is worth mentioning that all authors are bilingual in Somali and English.

Table 1
Sample of semi-structure interview guide.

Interview guide questions
1. From your own definition, how do you define health policy?
2. In your opinion, what role nurses play when it comes to health policy development?
3. From your perspective, are you pleased with how health policy is formulated, and implement? Explain
4. Based on your knowledge and experience, do you think nurses are part of or should be part of health policy development? If no, please explain (probing….)
5. From your perspective, what are the challenges or barriers that make nurses not to be part of health policy in Somaliland: individual level, institutional level and national level? probing
6. What we can do to improve and increase involvement of nurse in the health policy?

Table 2
Demographic information of the study participants (n = 13).

Respondents	Number (*N* = 13)
Age (years)	
a. 20–30	**2**
b. 31–40	**5**
c. 41–51	**6**
Gender	
a. Female	**8**
a. Male	**5**
Education	
a. Bachler's	**0**
b. Master level	**12**
c. PhD	**1**
Year of experience	
a. 5–25 years	**9**
b. 26–45 years	**4**
Workplace	
a. Governmental bodies	**4**
b. Universities	**7**
c. Non-governmental organizations	**1**
d. Professional association	**1**

2.5. Data analysis

The data were analyzed manually using the inductive reflexive thematic analysis technique (Braun & Clarke, 2022). The first stage of analysis was familiarization. Whereby the first and third authors read interview transcripts verbatim several times to understand the content and become immersed in the data. Second, all the transcripts related to the aim were extracted and condensed. In stage three, the condensed text was given headings (code). In stage four, similar codes were grouped into subthemes. The subthemes were divided based on their different contents and subthemes with similar content were grouped together to generate a main theme. (See Table 3, themes and subthemes identified). The first and third authors analyzed the data independently, and then regular meetings were held to discuss and come to a consensus. For the inter-coder reliability check, UI, and JE read through the original transcripts to ensure credibility and consistency. The theme and subthemes were discussed between the authors without disagreement and consensus was met. The authors contributed to all the steps of the data analysis and used reflexivity throughout the process to ensure that our analysis was grounded in the data at a manifest level. The study's research team was composed entirely of nurses, except for two midwives. Two authors served dual roles, as they work and live in the Global North while leading projects in the Global South. The other two authors were based in Somaliland, representing different spatial and occupational perspectives. Questions and reflections were discussed while analyzing the data and formulating the results.

Table 3
Theme and sub-theme.

Transforming nursing through education, leadership and systemic advocacy	Navigating the barriers to nursing education and equity	Challenges in the proliferation of and policy engagement
		Ambiguity and under compensation in nursing
	Building and strengthening a nursing ecosystem: education, leadership and advocacy	Empowering nurses in health policy development
		Strengthening nurses at the leadership level

2.6. Trustworthiness

Trustworthiness of this study is discussed using Lincoln and Guba's criteria for qualitative rigor, namely credibility, dependability and confirmability (Lincoln & Guba, 1985). Purposive sampling allowed the inclusion of a diverse group of stakeholders from academia, government and non-governmental organisations, which strengthened the study's credibility and dependability. Credibility was further enhanced through discussions among authors to refine and agree on the coding process. To ensure the confirmability, the authors remained attentive to potential personal biases and engaged in reflexive practices to support transparency and objectivity. In addition, the inclusion of direct participant quotations linkage between the data and the identified themes.

2.7. Ethical consideration

Ethical clearance was obtained from the University of Hargeisa (DRCS/60/01/2023) and the Ministry of Health Development (MOHD/DG: 2/307/2023). All the participants were given oral and written information about the study and were informed that they could withdraw at any time without any consequences.

3. Findings

In total, 14 participants were interviewed; eight participants were women, and the other five were men. Ten participants had nursing backgrounds, while the others had backgrounds in other health professional fields but held nursing leadership positions (see Table 2).

The main identified theme; Transforming nursing through education, leadership and systemic advocacy and its subthemes; (1) navigating the barriers to nursing education and equity and (2) building and strengthening a nursing ecosystem: education, leadership and advocacy are explained in Table 3. To illustrate the analysis, the themes detected from the interviews were analysed using the main theme and its subthemes.

3.1. Transforming nursing through education, leadership and systemic advocacy

This was the main theme identified from the analysis, and it was classified into two subthemes: (1) navigating the barriers to nursing education and equity and (2) building and strengthening a nursing ecosystem: education, leadership and advocacy.

3.1.1. Navigating the barriers to nursing education and equity

This sub-theme offers reasons for nurses' lack of involvement in health policy development: (1) the absence of regulations on nursing education due to the lack of accredited nursing schools, and (2) nurses' difficult work circumstances, which prevent them from pursuing their careers or building the capacity to be involved in decision-making roles (Table 1).

3.1.1.1. Challenges in the proliferation of non-accredited schools and policy engagement. The participants indicated that Somaliland's nursing education regulatory system was weak and poorly controlled. Many nurses graduated from private schools or colleges without proper regulations and accreditations, which resulted in the nursing profession being perceived as less valuable than other professions.

> *First, we need to ask ourselves: Do we need qualified nurses or a large number of nurses?……., you see numerous colleges that are open everywhere and produce graduates in just six months*. (Participant 9)

Most nurses work in clinical settings, but nurse's effectiveness is often limited, which is primarily due to them being exposed to insufficient curricula, non-accredited private schools and unqualified instructors. Consequently, when nurses transition into clinical practice, their performance may not meet expectations. Furthermore, they lack role models within the healthcare system to support their professional growth.

Moreover, the participants highlighted a hierarchical issue within the nursing profession: nurses operate equally regardless of qualification. For example, auxiliary nurses, despite not having much formal education, can hold positions of authority over highly educated nurses (BSc or MSc). This situation can result in a power imbalance in clinical settings, where the auxiliary nurses' role as ward leaders may prevent highly educated nurses from exercising their knowledge and skills effectively, thereby leading to challenges in collaboration and professional development. The highly educated nurses are thus left demotivated, powerless, and under confident. This situation also raises concerns about the structure of nursing roles, the importance of clearly delineating responsibilities and the need for empowering and supporting all nursing staff to ensure optimal patient care and professional growth.

> *You will see that auxiliary nurses are in charge of wards, while BSc or MSc are just bedside nurses who can't correct the auxiliary nurses because they don't have the power to do so*. (Participant 5)

The absence of a clear hierarchy in the healthcare system, the participants expressed, mainly stemmed from having no well-defined job descriptions, effective supervision, quality monitoring and accountability this situation generates various issues, including insufficient clarity about roles, inadequate oversight and failure to evaluate and improve quality of nurses. In addition, despite pursuing further education or specialised training, nurses are considered just nurses, with limited opportunities for career growth. Consequently, no system exists to hold individuals accountable for their actions or to motivate or discipline them as needed.

> *In my experience, I had a nursing diploma when I started my first job, I later pursued a bachelor's degree, after my master's degree. However, despite all these changes in my educational background, my salary, my position and my title remained the same.* (Participant 10)

The participants asserted that nurses are not sufficiently involved in managerial decision-making, which limits their ability to influence patient care policies and operational effectiveness. They also emphasised that, due to the absence of nurses in top-level positions, they cannot participate in decision-making. Moreover, non-nursing decision-makers assume roles typically associated with nursing, which excludes nurses from the decision-making process and from contributing to health policy development.

> *I think that nurses do not play a role in health policy development because they are not in decision-making positions*. (Participant 12)

3.1.1.2. Ambiguity and under compensation in nursing. Nurses operate in high-pressure and unsupportive environments and receive little assistance from hospital administration and policymakers. This inadequate support contributes to poorly organise clinical settings that lack essential resources for effective care, mainly because of the absence of clear guidelines and policies.

> *The clinical setting is not well prepared, and the environment is not useful because there are no guidelines or policies—nothing*. (Participant 12, University Lecturer)

The participants also highlighted that they perceived a lack of adequate support from their nursing association, suggesting a disparity in resource allocation and focus. The association appeared to prioritise and empower midwives due to the influx of international funding directed towards midwifery. The nurses thus felt marginalised and overlooked and caught in a professional limbo between the distinct roles and recognised contributions of midwives and physicians. Moreover, this imbalance left the nurses feeling unsupported and less valued by their own professional body.

> *The Somaliland Nursing and Midwifery Association is just for midwives, but they will tell you that it's for both nurses and midwives. To be honest, they are only for midwives, in my opinion*. (Participant 8)

Moreover, the nurses are often in survival mode, as they struggle to meet their basic needs with their low salaries. Some participants and their colleagues work three shifts to compensate for their low salaries, which hinders them from thinking about or pursuing a top-up degree or building their capacities.

> *Somaliland nurses receive insufficient salaries that are lower than those of other nurses around the world. This forces them to work three shifts*. (Participant 4)

3.1.2. Building and strengthening a nursing ecosystem: education, leadership and advocacy

This overarching theme addresses the factors that can facilitate nurses' engagement in health policy development in the future, as well as the nurses' participant recommendations. Some of these factors include nurses occupying leadership positions within their public educational institutions and robust leadership from associations that support nursing schools.

3.1.2.1. Empowering nurses in health policy development. The participants identified factors that could facilitate nursing engagement in health policy can play pivotal roles not only in healthcare settings but also as key leaders in nursing public education institutions. This suggests that nurses, through their expertise and clinical experience, could be at the forefront of shaping the direction, policies and quality of nursing education.

> *I think that nurses who graduated from public nursing health institutes possess the knowledge and skills needed to effectively integrate into and contribute to the health system and policy development*. (Participant 8)

The participants also indicated that a national nursing curriculum developed by nurses could foster an informed, well-equipped and empowered nursing workforce ready to participate in policy-making.

Furthermore, the participants agreed that all the public nursing schools in Somaliland are led by highly competent and educated nurses who possess the capacity, knowledge and experience to lead. We have nurses leading nursing departments in major universities, who can be

actively involved in policy formation.

Somaliland nurses also have their own associations that support public nursing schools. Nurses can utilise their association to advocate for their interests, promote the nursing profession and represent nurses. By empowering themselves, they can use their collective voices to influence health policies and participate in the decision-making process.

> *I think we're lucky to have a nursing association in Somaliland with long experience and sustainability, as neighbouring countries don't have a nursing association.* (Participant 3)

The participants highlighted that having a nursing association is one of the key strengths of Somaliland nurses, as it can help them gain the representation needed to engage in the health policy development process if they choose to be involved and pay the membership fee.

3.1.2.2. Strengthening nurses at the leadership level. The participants mentioned several factors that could encourage nurses to become involved in health policy development. They recommended that nursing schools should strengthen postgraduate nursing education by offering different nursing specialties, such as leadership and management at the master's level, through the support of a professional nursing association. They also underscored the importance of establishing a nursing council at the ministry level, which currently does not exist, to represent nurses at the national level and enable their participation in policies that affect the nursing profession.

> *we have a great number of nurses, but we need a well-planned master's program for leadership and management and need this at the ministry level. And meanwhile, the association should do its job parallel to this.* (Participant 12)

Some participants stressed the importance of adopting a top-down approach to help nurses attain suitable decision-making positions, as the bottom-up approach has not been effective in recent decades.

> *What are missing are strong leadership roles at the top, particularly at the ministry level.* (Participant 7)

Most participants emphasised the necessity of establishing an office within the Ministry of Health to advocate for nurses. The first step should be to appoint a representative for nurses at the national level, possibly through a dedicated position in the ministry, similar to the role of director of nurses in Somaliland.

4. Discussion

This study revealed an important gap in the Somaliland health administration: Nurses encounter organisational and systemic barriers that exclude them from health policy development. Their expertise is repeatedly ignored, preventing them from participating in the decision-making process and refusing those seats at the leadership table where important decisions are made. This finding aligns with theoretical frameworks of leadership in nursing which argues the need for leadership training programs that are context sensitive, evidence-based and adaptable to the realities of nursing practice (Charalambous, 2023).

One of the barriers identified is the proliferation of non-accredited nursing schools, which affects the academic preparation and competency of graduating nurses. This decline in academic quality not only weakens nurses' confidence in their clinical skills but also give a justification to the other decision-makers to exclude nurse from leadership positions. Consequently, nurses are absent from the policy development process in decision-making roles, and this absence creates a vacuum quickly filled by other professionals, mainly doctors, who take on the nursing role an imposition that not only interferes with the nurses professional scope but also breaks the organisational hierarchy system. Nurses are thus left to navigate unsupportive workplaces without appreciation and with low salaries, which validates their relegation within the health system. For instance, the limited involvement of nurses in policy development observed in this study reflects a deficiency in advocacy, which is the main domain of the WHO framework. Applying the WHO framework suggests that structured leadership development programs integrating mentorship, policy engagement training, and ethical leadership modules could address these gaps and enhance nurses' capacity to contribute to health system strengthening (World Health Organization, 2021).

Despite nurses facing such barriers to engaging with policy development, various facilitating factors signal the profession's unexploited potential. Developing academically qualified nurses and establishing national nursing associations and nurse-led public institutions demonstrate competency and dedication to evolving the nursing profession. These factors not only challenge the perspective of exclusion but also provide a foundation for encouraging nurses to regain their professional sway, design policies and assume strategic positions within the healthcare system.

To the best of our knowledge, ours is the first study that explored the perspectives of academia and government and non-governmental organisations about nurses' involvement in health policy in Somaliland. We detected a significant challenge in the nursing education regulatory system, which seems poorly controlled. The absence of effective regulations is a main barrier that restricts nurses from being involved in the health policy-making process. To successfully involve them, strong and well-prepared education programmes fulfilled by accredited and well-regulated nursing institutions are necessary (Wheeler et al., 2022). Such programmes can equip nurses with the knowledge, confidence and skills needed to navigate the health system and maintain their roles in policy-making. In the current system, many nurses graduate from private schools or colleges that lack proper regulation and accreditation. This subverts nursing's professional standing and leads to the perception that it is less valuable than other fields, thus further excluding nurses from being involved in policy development. Examining these gaps is vital to encourage nurses to participate in health governance and policy discussions. AS per similar findings from Ghana, 65 participants agreed that the lack of educational preparation is one of the reasons preventing nurses from being involved in health policy development (Alhassan et al., 2020). Moreover, Hajizadeh (2021) found that inadequate knowledge, skills and experience in health policy development is one of the main barriers to nurses' involvement in health policy development. Moreover, Keys (2014) found that the nurses who did not acquire formal educational leadership were unable to hold leadership positions. Evidently, it is important for nursing schools to prepare nursing students to become involved in future health policy development and to influence the structure and financing of the healthcare system.

Furthermore, the participants highlighted underpayment as a core challenge, with nurses receiving the lowest incentives among healthcare providers in Somaliland. Many nurses have to work multiple shifts just to meet basic living expenses, and this substantially limits their chances of pursuing higher education and advancing their careers. This disparity not only affects nurses but has wider effect on workforce, and health system performance. This finding is consistent with Bourgault et al. (2022), who found that insufficient salaries combined with high cost of living pushes nurses into a state of financial survival, leaving them with a limited capacity to pursue further education or professional advancement. Moreover, it was reported that over 70% of nurses are underpaid, with nearly half of them considering leaving the profession due to financial hardships. Such working conditions hinder nurses from increasing their capacity, ultimately widening their skill gaps (Royal College of Nursing, 2024).

A particular concern reported by the participants is that the absence of nursing leadership in healthcare has led to nurses being excluded from health policy involvement. Nurses are systematically excluded from leadership roles not because they lack qualifications but because other healthcare providers, mainly doctors, occupy nursing positions this practice stems from the belief that physicians possess more

expertise, granting them authority that affects the nursing profession. Such a dynamic excludes nurses and undermines essential nursing insights, ultimately diminishing the quality of patient care and safety. These findings are guided by a shared governance model, which argues that the lack of a standard leadership system excludes nurses from policy involvement and limits the influence of nurses' perspectives on decision-making, thereby affecting the quality of care and the health system's performance (Charalambous, 2023). A study conducted in Kenya found that the main barrier Kenyan nurses faced regarding health policy involvement was an interprofessional hierarchy, in which physicians were the dominant policy decision-makers in the health sector (Juma et al., 2014). Many nurses work as leaders, but unfortunately, they are not in the right leadership positions. Studies have revealed that limited organisational leadership opportunities for nurses are one of the barriers to their personal and career development (Hughes, 2018; Keys, 2014). In addition, Downey et al. (2011) found that nurses held unofficial leadership roles, working behind the scenes of wards/departments, and were described as the hidden treasure of informal leaders in the healthcare sector. An opportunity to participate would bring in experience; thus, a lack of involvement would lead to a lack of experience. Nurses must understand the importance of their role and re-evaluate it in terms of health policy development. To achieve this, Hughes (2018) suggested purposefully developing leaders through mentorship and succession plans to help eliminate nursing challenges related to the lack of leadership positions, as revealed by this study.

In Somaliland, the Ministry of Health is responsible for developing health policy through a top-down approach, which limits nurses from engaging directly in policy development, as this exclusion arises from their absence in high-ranking positions and participation only during policy implementation. As highlighted by the participants, this organisational gap hinders nurses from moving vertically within the health system, regardless of their qualifications. A study conducted in Kenya found that the main barrier faced by nurses in health policy development involvement is the hierarchy and structure of the health system, which prevents nurses from engaging with the health policy process (Juma et al., 2014). Similarly, Keys (2014) identified that nurses have limited organisational opportunities to move vertically to higher positions within the healthcare system. Moreover, several studies have highlighted the lack of supportive organisational structures as a main barrier for nurses to engage in the health policy development process (Safari et al., 2020). Enhancing top-down planning and decision-making is thus a significant aspect nurses should focus on. This improvement will help amplify the voices of those with less authority within the hierarchy.

One of the aims of this study was to uncover opportunities that empower nurses to play a more active role in health policy development in the near future. This study highlighted an important set of enabling factors that could improve nurses' involvement in health policy development nurses. Notably, participants emphasis that nurse, are not only integral to healthcare services but also lead all public nursing institutes in Somaliland. This leadership presence is evidence of nurses' capacity to hold leadership roles and allows them to design and participate in health policies. By elevating nurses to policy-shaping positions, the health system can benefit from grounded, practitioner-informed insights that reflect the frontline realities. This finding aligns with Adeyemi et al. (2022), according to which nurses in educational and advisory roles in countries such as the United Kingdom, Australia and the United States play a critical role in shaping health policies supported by legislative backing and organisational management. Similar findings have indicated that nurse educators are crucial in promoting policy awareness and advocacy among students, thereby nurturing the development of future nurse leaders involved in policy-making. This study thus suggests that unlocking this potential requires a deliberate shift towards recognising and utilising nurses' leadership capacities beyond traditional bedside care (Aminet al., 2025). The results of this study imply that having a national nursing association is an asset that nurses have that can be utilised to advocate for nursing profession rights and facilitate nurses to be in decision-making positions. A similar study found that 63 participants considered membership in a nursing profession association to enhance nurses' involvement in health policy development (Catallo et al., 2014). This result aligns with other studies that have stated that membership in associations enhances political behaviours, such as voting, mobilising resources, being familiar with policy makers and writing papers to policy makers (Boswell et al., 2005). Associations can thus improve the nursing role through advocacy, and actively participating in the programmes of professional associations may help nurses develop their political acumen. Moreover, the significance of providing nurses with opportunities to gain experience in being involved in health policy development to make their voices heard is also evident.

Furthermore, the participants emphasised the importance of establishing a nursing council at the ministry level, which currently does not exist. Such a council would provide representation for nurses at the national level and increase nurses' influence on health policy decisions. Evidence has shown that nursing councils function as a significant mechanism for protecting nurses' professional interests and ensuring their influence within the health system (Gumede, 2020). Similarly, the WHO has stressed the importance of an effective nursing regulatory body as a keystone for improving nursing leadership and policy involvement worldwide (World Health Organization, 2020). Creating such a council would be a vital step towards empowering nurses within the health system and improving their capacity to participate in national health policies.

4.1. *Implications for practice, education and research*

The health policy development process must include all stakeholders, including nurses, who hold positions related to health policy development. Nurses must lobby and create a health system structure that can facilitate their involvement in health policy development. In addition, the Somaliland Nursing and Midwifery Association must develop courses that address political content, leadership and management for nurses. In this study, we identified the barriers and opportunities for involvement in health policy development in Somaliland's largest city, Hargeisa. Similar studies could be conducted in other regions of the country with larger sample sizes.

4.2. *Strengths and limitations*

This study was purposive in nature, engaging various stakeholders, which enabled the triangulation of perspectives, which can be considered a strength. A limitation of this study is the inclusion of relatively small group of nurses working in clinical, academic, and administrative roles, as well as policymakers and individuals in leadership positions in Somaliland. Due to the limited sample size and the specific context of a health system undergoing reconstruction, the findings are not directly transferable. However, the results identify structural and professional factors that may be relevant to other low-resource or post-conflict settings where nursing education and health systems are being rebuilt. Additionally, the interview data were rich, but the study was conducted in one region and may not represent all regions of Somaliland. Therefore, caution should be taken when transferring the findings to other contexts.

5. Conclusion

The findings highlight several barriers to Somaliland nurses' involvement in health policy development that have been identified in other studies in other contexts, including unregulated nursing education, other healthcare providers occupying nursing roles, the absence of a hierarchy system in the healthcare system, the lack of a supportive environment and low salaries. Moreover, this study revealed that academically qualified nurses, national nursing associations, and nurses-

led public institutions demonstrate the competency and dedication of nurses to improve the nursing profession, which can, in turn, enhance and advocate nurses' roles in the healthcare system. These elements challenge the dominant narrative of exclusion and develop a foundation that empowers nurses to regain their professional sanctions to participate in policy development and take on essential leadership positions within the healthcare system. Our findings should be thus considered in terms of whether nurses' contributions to the current health policy development process will change. To facilitate nurses' involvement in health policy development, policy makers should initiate a process in which nurses' perspectives reach national decision-makers. To this end, nursing leaders can be intentionally developed through mentorship and succession plans, which will help eliminate the current nursing challenges. Furthermore, nursing associations should address and implement strategies for improving and strengthening nursing capacity in policy-making and advocate nursing representation at the national level.

CRediT authorship contribution statement

Umulkhayr Mohamed Ismail: Writing – review & editing, Writing – original draft, Visualization, Validation, Resources, Project administration, Methodology, Investigation, Funding acquisition, Formal analysis, Data curation, Conceptualization. **Hodan Mohamoud Osman:** Writing – review & editing, Validation, Conceptualization. **Jama Ali Egal:** Writing – review & editing, Validation, Methodology, Formal analysis, Conceptualization. **Fatumo Osman:** Writing – review & editing, Writing – original draft, Supervision, Methodology, Conceptualization.

Declaration of competing interest

The authors declare the following financial interests/personal relationships which may be considered as potential competing interests: Umulkhayr Mohamed Ismail reports financial support was provided by UK Research and Innovation. If there are other authors, they declare that they have no known competing financial interests or personal relationships that could have appeared to influence the work reported in this paper.

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Data availability

Data are not publicly available due to ethical restrictions, but anonymized excerpts may be shared with the editor and reviewers upon request.

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